Green Zone MO Guide

Right patient. Right zone. Right time.

HKL Emergency & Trauma Department

Open the app
Every pathway and rule, step by step

Patients move one way

MTS level 4 and 5 patients, from triage to discharge

1	2	3	4	5
Triage	**Secondary triage**	**You, the MO**	**Red Box**	**Treatment room (PPP)**
MTS level 4 or 5	ECG, irrigation, dressing, only if needed	Fill in the green slip, attach the queue number	The slip goes in; the patient waits in the tent	Calls the number, does each order, documents it

Results pending, observation or referral

PPP takes the patient to the CCA (Chair Centric Area) and hands over to Yellow Zone Extension staff, who record vital signs on the slip and send the patient back to you.

To finish

Results back: call the queue number again. Attach the slip to the BHT, complete the notes, give a TCA from the appointment flow chart and discharge advice.

Three rules to keep

Never waits outside
Bring them inside

Elderly **NSTEMI or ACS** **Stat drugs and monitoring**

- Seat them in the CCA, the rooms in front of R2, the R2 corridor or any zone with space
- The designated MA and HO there start management and monitoring
- No space: tell the EP in charge

Admit, discharge, transfer
Reassess first

- Reassess every patient; review every blood result and image
- Came within 24 hours: ask the TL, a master student or the EP to look at the case
- CT brain and going home: EP review, then an EMWATCH or EMACC appointment

When it stalls
Call the EP in charge

- Several disciplines and no disposition after a long wait: inform the EP
- External referral: discuss with the EP before accepting
- Pending at the end of your shift: hand over to the next MO and document who

Four more rules

One-way flow: no repeats after secondary triage

- Do not repeat the ECG, dressing or vital signs later unless they were missed
- An ECG you ask for in the Green Zone is done in the Procedure Room: do not send the patient back to secondary triage

Sources: HKL ED one-way procedure flow (Green Zone); KKM MTS announcement, Jul 2026.

Down-triage: the EP allows it

- Examine the patient properly first, then document it on the triage form

Source: memo HKL/JK/98/188/3(22)22, 4 Oct 2022. EP rule: to be confirmed.

IV antibiotics when no bed is free

- Look for a bed first in Yellow Obs, ISO or YZ Ext, and pass the patient over to the zone in-charge
- If none is free, the patient can have IV antibiotics on a chair in the CCA, with one of the four allowed antibiotics only: Augmentin, cefuroxime, ceftriaxone or Unasyn

Source: IV antibiotic memo, 30 Jun 2026.

When the primary team is delayed

- The EP may decide the admission within 4 hours if the primary team is delayed

Sources: KKM MTS announcement, Jul 2026; memo HKL/JK/98/188/3(17)23, 1 Sep 2023.

Numbers to know

EMACC follow-up
ext 7040
Via 03-2615 5555, MO in charge
Check: 2018 SOP

MAU mobile team
ext 6682 / 6684
After hours: MAU 1, 2 and 3 wards
Check: 2022 flow chart

IJN hotline
03-2617 8996
ACS meeting the referral criteria

Neuro Rehab Clinic
ext 7099
Mild TBI, normal CT, PTA over 4 h
Check: current directory

MAU FAST clinic
MHKL level 2
8 am–4:30 pm; CAP, CURB-65 0–1

Sources: Clinical Pathway in Emergency for MO HKL (ebook by the HKL Emergency Physicians); Green Zone memos 2022 to 2026; EMACC SOP 2018; MAU flow chart, 18 Oct 2022; HKL ED one-way procedure flow (Green Zone); down-triage memo, 4 Oct 2022; IV antibiotic memo, 30 Jun 2026; KKM MTS announcement, Jul 2026. A reference, not a substitute for clinical judgement: follow current department guidance. No patient data.

Pathways, follow-up and referrals

Green Zone MO Guide

The 23 ebook pathways

Admit · Observation · Discharge · Discharge advice

Listed by page in Clinical Pathway in Emergency for MO HKL. The colours are the ebook's outcome boxes.

5–9	Acute behavioural disturbance
10–11	Abdominal pain
12–14	Acute coronary syndrome and Obsess protocol
15–16	Anaphylaxis
17–18	Acute gastroenteritis (adult)
19–23	Bronchial asthma
24	Biliary disease
25–26	Mild traumatic brain injury
27	Community acquired pneumonia
28–29	CVA and TIA
30	Acute cellulitis
31–32	Dengue fever
33–35	Deep vein thrombosis
36–37	Heart failure
38–40	Hypertensive urgency
41–42	Hyperglycaemia
43–44	Hypoglycaemia
45–50	Pain management
51–52	Paracetamol overdose
53–54	Renal colic
55–56	Supraventricular tachycardia
57–58	Syncope
59–63	Acute scrotal pain

EMACC follow-up

`ext 7040` · Green Zone, Mon–Fri 9 am–4 pm · first review within 1 week

For adults 18+ being discharged, with no own-clinic appointment within 14 days, a case below, and no admission needed.

Case	Review in EMACC
Hypertensive urgency	Within 1 week for BP review; renal profile if started on captopril
Low-risk chest pain after OBSSES	Within 2 weeks: symptoms, repeat ECG, EST result; aspirin 150 mg OD if no contraindication; EST within 1 week, or refer to PC if EST is contraindicated
Uncontrolled DM: sugar over 11, not DKA or HHS, no admission needed	Within 1 week if there is no timely primary clinic appointment
Heart failure improved with ED treatment, no admission needed	Within 1 week; repeat renal profile if frusemide or an ACE inhibitor was started; suspected new HF: also refer to PC
Renal colic, improved, normal renal profile	First episode with a normal KUB X-ray: EMACC review. Recurrent: KUB ultrasound, then EMACC reviews symptoms and result
Non-specific abdominal pain: 7 days or less, no diagnosis, normal workup, improved	Within 3 days; discuss every case with the EP in charge
Trauma: up to 2 uncomplicated rib fractures, undisplaced closed fracture, minor dislocation, suspected fracture with a normal X-ray, soft tissue injury with marked swelling, large laceration (especially in diabetics)	Within 3 days for a wound check and pain review; repeat X-ray if needed; MC until the EMACC appointment
Blood results to review, such as TFT	As arranged

Asthma Zone

1. **Triage level given at primary triage?** → Level 2 or 3: **Critical zone, R1**
2. **Level 4 or 5** **Asthma card and a 5000 queue number**
3. **Asthma Treatment Room (PPP)**
   - Secondary assessment; uptriage at once if unstable
   - Vital signs and PEFR before and after treatment
   - Reassess at 15–20 minutes
4. **Resolved after the inhaler cycle?** → Not resolved: **Uptriage to R1**
5. **Resolved** **Health education, then the card comes to your room**
6. **You re-examine the patient. Stable?** → Not stable: **Uptriage to R1**

Stable **Discharge (you, the Green Zone MO)**

- Give the balance of the MDI the patient used, with the spacer
- Prednisolone 30 mg once daily for 5 days
- Salbutamol MDI 4 puffs every 4–6 hours for 1 day, then 2 puffs as needed
- Do not share the MDI; bring your own spacer next time; come back at once (TCA stat) if the attack recurs

Source: Asthma memo, Nov 2025. Discharge prescription: department BA/COPD protocol, 27 Mar 2020.

Refer to

- **Joint dislocation**, shoulder too: the on-call Orthopaedic team, not the clinic.
- **GI bleed:** medical cause, Gastroenterology; surgical cause, General Surgery. **Cholecystitis** without jaundice: General Surgery. **Jaundice:** Gastroenterology.
- **Medical ward:** refer to the Medical specialist or MO on call first.
- **Psychiatric relapse**, known history, no other medical problem: Psychiatry MO on call, no bloods or CT first.
- **Gout:** nearest Klinik Kesihatan; complex cases to Rheumatology.
- **Dengue from a KK**, warning signs only, normal CCTVR and vitals, platelets over 50,000, not morbidly obese: straight to the Observation Ward.
- **IPR (respiratory):** stable, no inotropes, Venturi 40% at most.

Sources: the ebook above; EMACC SOP (1 Feb 2018: check the extension and hours are current); Asthma memo, 13 Nov 2025; department BA/COPD protocol, 27 Mar 2020; referral memos and letters 2018 to 2026. A reference, not a substitute for clinical judgement. No patient data.